BASELINE → TREATMENT → FOLLOW-UP

TCHP: where scans and echoes sit against the six cycles

The week-12 echo is approximately before Cycle 5 only if cycles remain on time. Use elapsed time from the cardiac assessment when treatment is delayed. Breast imaging for surgical planning has an individual date; the map must not invent a universal midpoint scan.

Check	Before Cycle 1	Cycles 1–4 · weeks 0–9	Around Cycle 5 · week 12	Cycle 6 / surgery planning	Further HER2 treatment
Echo · heart pumping function	Baseline LVEF before treatment	Earlier if symptoms / abnormal result	Repeat at 12 weeks	Bring results into surgery / onward plan	Continue every 12 weeks; assess at HER2 completion
Blood count / renal / liver	Baseline; assess kidney function for carboplatin	Before each 3-week cycle	Before Cycle 5	Before Cycle 6; confirm recovery	According to the ongoing regimen
Breast imaging / response	Baseline breast imaging; staging scans if indicated	Clinical response at reviews; image for concern	Additional imaging only if indicated	Breast imaging for operative planning as agreed	Pathology guides postoperative treatment
CT / tumour markers	Staging only when indicated	No routine CT or marker test every cycle	No fixed Cycle 5 CT requirement	Further tests if clinically indicated	No routine CT / markers for symptom-free early-breast follow-up

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00722 v2 · official regimen · ESMO 2024 early breast cancer guideline · treatment and follow-up · ESC 2022 cardio-oncology guideline · cardiac surveillance and risk · ASCO · neoadjuvant breast therapy and response assessment

THE WHOLE TREATMENT JOURNEY

TCHP: before surgery and beyond

Neoadjuvant · HER2-positive early breast cancer

Six cycles before surgery are one part of the pathway. Surgery findings determine the treatment afterwards; hormone treatment is added only if the cancer is also hormone-receptor positive.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before treatment	TCHP · about 18 weeks	Surgery and pathology	After surgery	Longer-term follow-up
Cancer medicines NCCP 00722 · EARLY	Checks and access plan	TCHP every 3 weeks ×6	Pause for surgery; review response	HER2 treatment chosen from pathology; commonly to complete a year overall	Review after the planned course
Surgery EARLY · NEO	Discuss breast and lymph-node surgery	Review response and plan the operation	Lumpectomy OR mastectomy; lymph-node surgery as indicated	Recovery, wound and shoulder/arm review	Report new breast, arm or scar concerns
Radiotherapy EARLY	Discuss whether likely to be needed	Usually planned after chemotherapy and surgery	Decision uses operation and pathology	If indicated: may overlap HER2 treatment; team agrees timing	Review skin, fatigue and late effects
Endocrine / bone care EARLY · BONE · NOGG	Check ER/PR status	Not a substitute for TCHP	Plan if HR-positive	Endocrine treatment if indicated; assess bone protection	Endocrine treatment can continue for years; DEXA by risk
Blood tests NCCP 00722	Blood count, kidney and liver tests; GFR	Before every cycle	Preoperative checks arranged by surgical team	According to the new regimen	Only as clinically indicated
Heart checks NCCP 00722 · ESC	Baseline echo / LVEF	Echo about every 12 weeks	Carry the cardiac monitoring plan forward	Continue HER2 surveillance; assessment at completion	Risk-based follow-up; assess symptoms promptly
Cancer imaging NEO · EARLY	Breast imaging; staging if indicated	Clinical response review; imaging for concern or surgical planning	Pathology shows response	Plan surveillance of remaining breast	Annual mammography; no routine CT/markers if well

What each check is for

1 Heart ultrasound (echo)

Before HER2 therapy; every 12 weeks during it; at completion

Checks heart pumping function, even without symptoms.

Ask / act: Ask when the next echo is due and who arranges it. Report new breathlessness, swelling or palpitations.

Sources: NCCP 00722 · ESC

2 Response and operation planning

During neoadjuvant treatment and before surgery

Checks the local response and helps plan surgery.

Ask / act: Ask which breast imaging is planned and whether the result changes the operation.

Sources: NEO

3 After surgery: choose the next pathway

When pathology is available

Residual cancer and receptor status can change the postoperative medicines.

Ask / act: Ask for the name, purpose and planned duration of the next treatment.

Sources: NCCP 00722 · EARLY

4 Bone health, if endocrine therapy is prescribed

At the endocrine-treatment decision; then by risk

Assess fracture risk and whether a baseline DEXA bone-density scan is needed when starting an aromatase inhibitor or ovarian suppression/treatment-induced menopause. It may already have been done before this regimen.

Ask / act: Ask whether a DEXA result is already available and whether bone protection is recommended.

Sources: BONE · NOGG

5

Surveillance after early breast treatment

After completion of local treatment

Annual mammography of remaining breast tissue after treatment; the team sets the first date. The mastectomy side is not routinely mammographed. New symptoms need assessment between visits.

Ask / act: Report new symptoms without waiting for the next mammogram.

Sources: EARLY · BONE

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.